

they did not overtly consider these entities' rights under RFRA.

This is especially true in light of the basic requirements of the rulemaking process. Our precedents require final rules to “articulate a satisfactory explanation for [the] action including a rational connection between the facts found and the choice made.” *Motor Vehicle Mfrs. Assn. of United States, Inc. v. State Farm Mut. Automobile Ins. Co.*, 463 U.S. 29, 43 (1983) (internal quotation marks omitted). This requirement allows courts to assess whether the agency has promulgated an arbitrary and capricious rule by “entirely fail[ing] to consider an important aspect of the problem [or] offer[ing] an explanation for its decision that runs counter to the evidence before [it].” *Ibid.*; see also *Department of Commerce v. New York*, 588 U.S. 752, 802 (2019) (BREYER, J., concurring in part and dissenting in part); *Genuine Parts Co. v. EPA*, 890 F.3d 304, 307 (CA DC 2018); *Pacific Coast Federation of Fishermen's Assns. v. United States Bur. of Reclamation*, 426 F.3d 1082, 1094 (CA9 2005). Here, the Departments were aware that *Hobby Lobby* held the mandate unlawful as applied to religious entities with complicity-based objections. 82 Fed. Reg. 47799; 83 Fed. Reg. 57544–57545. They were also aware of *Zubik*'s instructions. 82 Fed. Reg. 47799. And, aside from our own decisions, the Departments were mindful of the RFRA concerns raised in “public comments and . . . court filings in dozens of cases—encompassing hundreds of organizations.” *Id.*, at 47802; see also *id.*, at 47806. If the Departments did not look to RFRA's requirements or discuss RFRA at all when formulating their solution, they would certainly be susceptible to claims that the rules were arbitrary and capricious for failing to consider an important aspect of the problem.¹² Thus, respondents' argument that the Departments

¹² Here, too, the Departments have consistently taken the position that their rules had to account for RFRA in response to comments that the rules would violate that statute. See Dept. of Labor, FAQs About Afford-

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erred by looking to RFRA as a guide when framing the religious exemption is without merit.

III

Because we hold that the Departments had authority to promulgate the exemptions, we must next decide whether the 2018 final rules are procedurally invalid. Respondents present two arguments on this score. Neither is persuasive.

A

Unless a statutory exception applies, the APA requires agencies to publish a notice of proposed rulemaking in the Federal Register before promulgating a rule that has legal force. See 5 U.S.C. §553(b). Respondents point to the fact that the 2018 final rules were preceded by a document entitled “Interim Final Rules with Request for Comments,” not a document entitled “General Notice of Proposed Rulemaking.” They claim that since this was insufficient to satisfy §553(b)’s requirement, the final rules were procedurally invalid. Respondents are incorrect. Formal labels aside, the rules contained all of the elements of a notice of proposed rulemaking as required by the APA.

The APA requires that the notice of proposed rulemaking contain “reference to the legal authority under which the rule is proposed” and “either the terms or substance of the proposed rule or a description of the subjects and issues involved.” §§553(b)(2)–(3). The request for comments in the 2017 IFRs readily satisfies these requirements. That request detailed the Departments’ view that they had legal

able Care Act Implementation Part 36, pp. 4–5 (2017) (2016 Request for Information); 78 Fed. Reg. 39886–39887 (2013 rule); 77 Fed. Reg. 8729 (2012 final rule). As the 2017 IFR explained, the Departments simply reached a different conclusion on whether the accommodation satisfied RFRA. See 82 Fed. Reg. 47800–47806 (summarizing the previous ways in which the Departments accounted for RFRA and providing a lengthy explanation for the changed position).

authority under the ACA to promulgate both exemptions, 82 Fed. Reg. 47794, 47844, as well as authority under RFRA to promulgate the religious exemption, *id.*, at 47800–47806. And respondents do not—and cannot—argue that the IFRs failed to air the relevant issues with sufficient detail for respondents to understand the Departments’ position. See *supra*, at 671–672. Thus, the APA notice requirements were satisfied.

Even assuming that the APA requires an agency to publish a document entitled “notice of proposed rulemaking” when the agency moves from an IFR to a final rule, there was no “prejudicial error” here. § 706. We have previously noted that the rule of prejudicial error is treated as an “administrative law . . . harmless error rule,” *National Assn. of Home Builders v. Defenders of Wildlife*, 551 U.S. 644, 659–660 (2007) (internal quotation marks omitted). Here, the Departments issued an IFR that explained its position in fulsome detail and “provide[d] the public with an opportunity to comment on whether [the] regulations . . . should be made permanent or subject to modification.” 82 Fed. Reg. 47815; see also *id.*, at 47852, 47855. Respondents thus do not come close to demonstrating that they experienced any harm from the title of the document, let alone that they have satisfied this harmless error rule. “The object [of notice and comment], in short, is one of fair notice,” *Long Island Care at Home, Ltd. v. Coke*, 551 U.S. 158, 174 (2007), and respondents certainly had such notice here. Because the IFR complied with the APA’s requirements, this claim fails.¹³

B

Next, respondents contend that the 2018 final rules are procedurally invalid because “nothing in the record signal[s]”

¹³ We note as well that the Departments promulgated many other IFRs in addition to the three related to the contraceptive mandate. See, *e.g.*, 75 Fed. Reg. 27122 (dependent coverage); *id.*, at 34538 (grandfathered health plans); *id.*, at 37188 (pre-existing conditions).

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that the Departments “maintained an open mind throughout the [post-promulgation] process.” Brief for Respondents 27. As evidence for this claim, respondents point to the fact that the final rules made only minor alterations to the IFRs, leaving their substance unchanged. The Third Circuit applied this “open-mindedness” test, concluding that because the final rules were “virtually identical” to the IFRs, the Departments lacked the requisite “flexible and open-minded attitude” when they promulgated the final rules. 930 F. 3d, at 569 (internal quotation marks omitted).

We decline to evaluate the final rules under the open-mindedness test. We have repeatedly stated that the text of the APA provides the “‘maximum procedural requirements’” that an agency must follow in order to promulgate a rule. *Perez*, 575 U. S., at 100 (quoting *Vermont Yankee Nuclear Power Corp. v. Natural Resources Defense Council, Inc.*, 435 U. S. 519, 524 (1978)). Because the APA “sets forth the full extent of judicial authority to review executive agency action for procedural correctness,” *FCC v. Fox Television Stations, Inc.*, 556 U. S. 502, 513 (2009), we have repeatedly rejected courts’ attempts to impose “judge-made procedur[es]” in addition to the APA’s mandates, *Perez*, 575 U. S., at 102; see also *Pension Benefit Guaranty Corporation v. LTV Corp.*, 496 U. S. 633, 654–655 (1990); *Vermont Yankee*, 435 U. S., at 549. And like the procedures that we have held invalid, the open-mindedness test violates the “general proposition that courts are not free to impose upon agencies specific procedural requirements that have no basis in the APA.” *LTV Corp.*, 496 U. S., at 654. Rather than adopting this test, we focus our inquiry on whether the Departments satisfied the APA’s objective criteria, just as we have in previous cases. We conclude that they did.

Section 553(b) obligated the Departments to provide adequate notice before promulgating a rule that has legal force. As explained *supra*, at 683–684, the IFRs provided sufficient notice. Aside from these notice requirements, the APA

mandates that agencies “give interested persons an opportunity to participate in the rule making through submission of written data, views, or arguments,” § 553(c); states that the final rules must include “a concise general statement of their basis and purpose,” *ibid.*; and requires that final rules must be published 30 days before they become effective, § 553(d).

The Departments complied with each of these statutory procedures. They “request[ed] and encourage[d] public comments on all matters addressed” in the rules—*i. e.*, the basis for the Departments’ legal authority, the rationales for the exemptions, and the detailed discussion of the exemptions’ scope. 82 Fed. Reg. 47813, 47854. They also gave interested parties 60 days to submit comments. *Id.*, at 47792, 47838. The final rules included a concise statement of their basis and purpose, explaining that the rules were “necessary to protect sincerely held” moral and religious objections and summarizing the legal analysis supporting the exemptions. 83 Fed. Reg. 57592; see also *id.*, at 57537–57538. Lastly, the final rules were published on November 15, 2018, but did not become effective until January 14, 2019—more than 30 days after being published. *Id.*, at 57536, 57592. In sum, the rules fully complied with “the maximum procedural requirements [that] Congress was willing to have the courts impose upon agencies in conducting rulemaking procedures.” *Perez*, 575 U. S., at 102 (quoting *Vermont Yankee*, 435 U. S., at 524). Accordingly, respondents’ second procedural challenge also fails.¹⁴

* * *

For over 150 years, the Little Sisters have engaged in faithful service and sacrifice, motivated by a religious calling

¹⁴ Because we conclude that the IFRs’ request for comment satisfies the APA’s rulemaking requirements, we need not reach respondents’ additional argument that the Departments lacked good cause to promulgate the 2017 IFRs.

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to surrender all for the sake of their brother. “[T]hey commit to constantly living out a witness that proclaims the unique, inviolable dignity of every person, particularly those whom others regard as weak or worthless.” Complaint ¶14. But for the past seven years, they—like many other religious objectors who have participated in the litigation and rule-makings leading up to today’s decision—have had to fight for the ability to continue in their noble work without violating their sincerely held religious beliefs. After two decisions from this Court and multiple failed regulatory attempts, the Federal Government has arrived at a solution that exempts the Little Sisters from the source of their complicity-based concerns—the administratively imposed contraceptive mandate.

We hold today that the Departments had the statutory authority to craft that exemption, as well as the contemporaneously issued moral exemption. We further hold that the rules promulgating these exemptions are free from procedural defects. Therefore, we reverse the judgment of the Court of Appeals and remand the cases for further proceedings consistent with this opinion.

It is so ordered.

JUSTICE ALITO, with whom JUSTICE GORSUCH joins, concurring.

In these cases, the Court of Appeals held, among other things, (1) that the Little Sisters of the Poor lacked standing to appeal, (2) that the Affordable Care Act (ACA) does not permit any exemptions from the so-called contraceptive mandate, (3) that the Departments responsible for issuing the challenged rule¹ violated the Administrative Procedure

¹The Health Resources and Services Administration (HRSA), a division of the Department of Health and Human Services, creates the “comprehensive guidelines” on “coverage” for “additional preventive care and screenings” for women, 42 U. S. C. §300gg–13(a)(4), but the statute is

Act (APA) by failing to provide notice of proposed rule-making, and (4) that the final rule creating the current exemptions is invalid because the Departments did not have an open mind when they considered comments to the rule. Based on this analysis, the Court of Appeals affirmed the nationwide injunction issued by the District Court.

This Court now concludes that all the holdings listed above were erroneous, and I join the opinion of the Court in full. We now send these cases back to the lower courts, where the Commonwealth of Pennsylvania and the State of New Jersey are all but certain to pursue their argument that the current rule is flawed on yet another ground, namely, that it is arbitrary and capricious and thus violates the APA. This will prolong the legal battle in which the Little Sisters have now been engaged for seven years—even though during all this time no employee of the Little Sisters has come forward with an objection to the Little Sisters’ conduct.

I understand the Court’s desire to decide no more than is strictly necessary, but under the circumstances here, I would decide one additional question: whether the Court of Appeals erred in holding that the Religious Freedom Restoration Act (RFRA), 42 U. S. C. §§ 2000bb to 2000bb–4, does not compel the religious exemption granted by the current rule. If RFRA requires this exemption, the Departments did not act in an arbitrary and capricious manner in granting it. And in my judgment, RFRA compels an exemption for the Little Sisters and any other employer with a similar objection to what has been called the accommodation to the contraceptive mandate.

jointly administered and enforced by the Departments of Health and Human Services, Labor, and Treasury (collectively Departments), see § 300gg–92; 29 U. S. C. § 1191c; 26 U. S. C. § 9833. The Departments promulgated the exemptions at issue here, which were subsequently incorporated into the guidelines by HRSA. See 83 Fed. Reg. 57536 (2018); *id.*, at 57592.

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I

Because the contraceptive mandate has been repeatedly modified, a brief recapitulation of this history may be helpful. The ACA itself did not require that insurance plans include coverage for contraceptives. Instead, the Act provided that plans must cover those preventive services found to be appropriate by HRSA, an agency of the Department of Health and Human Services. 42 U. S. C. § 300gg–13(a)(4). In 2011, HRSA recommended that plans be required to cover “[a]ll . . . contraceptive methods” approved by the Food and Drug Administration. 77 Fed. Reg. 8725 (2012). (I will use the term “contraceptive mandate” or simply “mandate” to refer to the obligation to provide coverage for contraceptives under any of the various regimes that have existed since the promulgation of this original rule.) At the direction of the relevant Departments, HRSA simultaneously created an exemption from the mandate for “churches, their integrated auxiliaries, and conventions or associations of churches,” as well as “the exclusively religious activities of any religious order.” 76 Fed. Reg. 46623 (2011); see 77 Fed. Reg. 8726. (I will call this the “church exemption.”) This narrow exemption was met with strong objections on the ground that it furnished insufficient protection for religious groups opposed to the use of some or all of the listed contraceptives.

The Departments responded by issuing a new regulation that created an accommodation for certain religious non-profit employers. See 78 Fed. Reg. 39892–39898 (2013). (I will call this the “accommodation.”) Under this accommodation, a covered employer could certify its objection to its insurer (or, if its plan was self-funded, to its third-party plan administrator), and the insurer or third-party administrator would then proceed to provide contraceptive coverage to the objecting entity’s employees. Unlike the earlier church exemption, the accommodation did not exempt these religious employers from the contraceptive mandate, but the Depart-

ments construed invocation of the accommodation as compliance with the mandate.

Meanwhile, the contraceptive mandate was challenged by various employers who had religious objections to providing coverage for at least some of the listed contraceptives but were not covered by the church exemption or the accommodation. In *Burwell v. Hobby Lobby Stores, Inc.*, 573 U.S. 682 (2014), we held that RFRA prohibited the application of the regulation to closely held, for-profit corporations that fell into this category. The Departments responded by issuing a new regulation that attempted to codify our holding by allowing closely held corporations to utilize the accommodation. See 80 Fed. Reg. 41343–41347 (2015).²

Although this modification solved one RFRA problem, the contraceptive mandate was still objectionable to some religious employers, including the Little Sisters. We considered those objections in *Zubik v. Burwell*, 578 U.S. 403 (2016) (*per curiam*), but instead of resolving the legal dispute, we vacated the decisions below and remanded, instructing the parties to attempt to come to an agreement. Unfortunately, after strenuous efforts, the outgoing administration reported on January 9, 2017, that no reconciliation could be reached.³ The Little Sisters and other employers objected to engaging in any conduct that had the effect of making contraceptives available to their employees under their insurance plans, and no way of providing such coverage to their employees without using their plans could be found.

²In the regulation, the Departments also responded to our holding in *Wheaton College v. Burwell*, 573 U.S. 958 (2014), by allowing employers who invoked the accommodation to notify the Government of their objection rather than by filing the objection with their insurer or third-party administrator. See 80 Fed. Reg. 41337.

³Dept. of Labor, FAQs About Affordable Care Act Implementation Part 36 (Jan. 9, 2017), <https://www.dol.gov/sites/dolgov/files/EBSA/about-ebsa/our-activities/resource-center/faqs/aca-part-36.pdf>.

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In 2017, the new administration took up the task of attempting to find a solution. After receiving more than 56,000 comments, it issued the rule now before us, which made the church exemption available to non-governmental employers who object to the provision of some or all contraceptive services based on sincerely held religious beliefs.⁴ 45 CFR § 147.132; see 83 Fed. Reg. 57540, 57590. (The “religious exemption.”) The Court of Appeals, as noted, held that RFRA did not require this new rule.

II

A

RFRA broadly prohibits the Federal Government from violating religious liberty. See 42 U. S. C. § 2000bb–1(a). It applies to every “branch, department, agency, [and] instrumentality” of the Federal Government, as well as any “person acting under color of” federal law. § 2000bb–2(1). And this prohibition applies to the “implementation” of federal law. § 2000bb–3(a). Thus, unless the ACA or some other subsequently enacted statute made RFRA inapplicable to the contraceptive mandate, the Departments responsible for administering that mandate are obligated to do so in a manner that complies with RFRA.

No provision of the ACA abrogates RFRA, and our decision in *Hobby Lobby*, 573 U. S., at 736, established that application of the contraceptive mandate must conform to RFRA’s demands. Thus, it was incumbent on the Departments to ensure that the rules implementing the mandate were consistent with RFRA, as interpreted in our decision.

B

Under RFRA, the Federal Government may not “substantially burden a person’s exercise of religion even if the bur-

⁴ A similar exemption was provided for employers with moral objections. See 45 CFR § 147.33 (2019).

den results from a rule of general applicability,” unless it “demonstrates that application of the burden to the person— (1) is in furtherance of a compelling governmental interest; and (2) is the least restrictive means of furthering that compelling governmental interest.” §§ 2000bb–1(a), (b). Applying RFRA to the contraceptive mandate thus presents three questions. First, would the mandate substantially burden an employer’s exercise of religion? Second, if the mandate would impose such a burden, would it nevertheless serve a “compelling interest”? And third, if it serves such an interest, would it represent “the least restrictive means of furthering” that interest?

Substantial burden. Under our decision in *Hobby Lobby*, requiring the Little Sisters or any other employer with a similar religious objection to comply with the mandate would impose a substantial burden. Our analysis of this question in *Hobby Lobby* can be separated into two parts. First, would non-compliance have substantial adverse practical consequences? 573 U. S., at 720–723. Second, would compliance cause the objecting party to violate its religious beliefs, *as it sincerely understands them*? *Id.*, at 723–726.

The answer to the first question is indisputable. If a covered employer does not comply with the mandate (by providing contraceptive coverage or invoking the accommodation), it faces penalties of \$100 per day for each of its employees. 26 U. S. C. § 4980D(b)(1). “And if the employer decides to stop providing health insurance altogether and at least one full-time employee enrolls in a health plan and qualifies for a subsidy on one of the government-run ACA exchanges, the employer must pay \$2,000 per year for each of its full-time employees. §§ 4980H(a), (c)(1).” 573 U. S., at 697. In *Hobby Lobby*, we found these “severe” financial consequences sufficient to show that the practical effect of non-compliance would be “substantial.”⁵ *Id.*, at 720.

⁵This is one of the differences between these cases and *Bowen v. Roy*, 476 U. S. 693 (1986). See *post*, at 727 (GINSBURG, J., dissenting) (relying on *Bowen* to conclude that accommodation was unnecessary). In *Bowen*,

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Our answer to the second question was also perfectly clear. If an employer has a religious objection to the use of a covered contraceptive, and if the employer has a sincere religious belief that compliance with the mandate makes it complicit in that conduct, then RFRA requires that the belief be honored. *Id.*, at 724–725. We noted that the objection raised by the employers in *Hobby Lobby* “implicate[d] a difficult and important question of religion and moral philosophy, namely, the circumstances under which it is wrong for a person to perform an act that is innocent in itself but that has the effect of enabling or facilitating the commission of an immoral act by another.” *Id.*, at 724. We noted that different individuals have different beliefs on this question, but we were clear that “federal courts have no business addressing . . . whether the religious belief asserted in a RFRA case is reasonable.” *Ibid.* Instead, the “‘function’” of a court is “‘narrow’”: “‘to determine’ whether the line drawn reflects ‘an honest conviction.’” *Id.*, at 725 (quoting *Thomas v. Review Bd. of Ind. Employment Security Div.*, 450 U. S. 707, 716 (1981)).

Applying this holding to the Little Sisters yields an obvious answer. It is undisputed that the Little Sisters have a sincere religious objection to the use of contraceptives and that they also have a sincere religious belief that utilizing the accommodation would make them complicit in this conduct. As in *Hobby Lobby*, “it is not for us to say that their religious beliefs are mistaken or insubstantial.” 573 U. S., at 725.

In reaching a contrary conclusion, the Court of Appeals adopted the reasoning of a prior Third Circuit decision holding that “‘the submission of the self-certification form’” required by the mandate would not “‘trigger or facilitate the provision of contraceptive coverage’” and would not make the Little Sisters “‘“complicit” in the provision’” of

the objecting individuals were not faced with penalties or “coerced by the Governmen[t] into violating their religious beliefs.” *Lyng v. Northwest Indian Cemetery Protective Assn.*, 485 U. S. 439, 449 (1988).

objected-to services. 930 F. 3d 543, 573 (2019) (quoting *Geneva College v. Secretary of U. S. Dept. of Health and Human Servs.*, 778 F. 3d 422, 437–438 (CA3 2015), vacated and remanded *sub nom.* *Zubik*, 578 U. S. 403).

The position taken by the Third Circuit was similar to that of the Government when *Zubik* was before us. Opposing the position taken by the Little Sisters and others, the Government argued that what the accommodation required was not materially different from simply asking that an objecting party opt out of providing contraceptive coverage with the knowledge that by doing so it would cause a third party to provide that coverage. According to the Government, everything that occurred following the opt-out was a result of governmental action.⁶

Petitioners disagreed. Their concern was not with notifying the Government that they wished to be exempted from complying with the mandate *per se*,⁷ but they objected to two requirements that they sincerely believe would make them complicit in conduct they find immoral. First, they took strong exception to the requirement that they maintain and pay for a plan under which coverage for contraceptives would be provided. As they explained, if they “were willing to incur ruinous penalties by dropping their health plans, their insurance companies would have no authority or obligation to provide or procure the objectionable coverage for [their] plan beneficiaries.”⁸ Second, they also objected to submission of the self-certification form required by the accommodation because without that certification their plan could not be used to provide contraceptive coverage.⁹ At

⁶ See Brief for Respondents in *Zubik v. Burwell*, O. T. 2015, Nos. 14–1418, 14–1453, 14–1505, 15–35, 15–105, 15–119, 15–191, pp. 35–41.

⁷ See Brief for Petitioners in *Zubik v. Burwell*, O. T. 2015, Nos. 15–35, 15–105, 15–119, 15–191, p. 45.

⁸ Brief for Petitioners in *Zubik v. Burwell*, O. T. 2015, Nos. 14–1418, 14–1453, 14–1505, p. 49.

⁹ Brief for Petitioners in *Zubik*, O. T. 2015, Nos. 15–35, 15–105, 15–119, 15–191, at 44.

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bottom, then, the Government and the religious objectors disagreed about the relationship between what the accommodation demanded and the provision of contraceptive coverage.

Our remand in *Zubik* put these two conflicting interpretations to the test. In response to our request for supplemental briefing, petitioners explained their position in the following terms. “[T]heir religious exercise” would not be “infringed” if they did not have to do anything “‘more than contract for a plan that does not include coverage for some or all forms of contraception,’ even if their employees receive[d] cost-free contraceptive coverage from the same insurance company.” 578 U. S., at 407–408. At the time, the Government thought that it might be possible to achieve this result under the ACA, *id.*, at 408, but subsequent attempts to find a way to do this failed. After great effort, the Government was forced to conclude that it was “not aware of the authority, or of a practical mechanism,” for providing contraceptive coverage “specifically to persons covered by an objecting employer, other than by using the employer’s plan, issuer, or third party administrator.” 83 Fed. Reg. 57545–57546.

The inescapable bottom line is that the accommodation demanded that parties like the Little Sisters engage in conduct that was a necessary cause of the ultimate conduct to which they had strong religious objections. Their situation was the same as that of the conscientious objector in *Thomas*, 450 U. S., at 715, who refused to participate in the manufacture of tanks but did not object to assisting in the production of steel used to make the tanks. Where to draw the line in a chain of causation that leads to objectionable conduct is a difficult moral question, and our cases have made it clear that courts cannot override the sincere religious beliefs of an objecting party on that question. See *Hobby Lobby*, 573 U. S., at 723–726; *Thomas*, 450 U. S., at 715–716.

For these reasons, the contraceptive mandate imposes a substantial burden on any employer who, like the Little Sisters, has a sincere religious objection to the use of a listed

contraceptive and a sincere religious belief that compliance with the mandate (through the accommodation or otherwise) makes it complicit in the provision to the employer's workers of a contraceptive to which the employer has a religious objection.

Compelling interest. In *Hobby Lobby*, the Government asserted and we assumed for the sake of argument that the Government had a compelling interest in "ensuring that all women have access to all FDA-approved contraceptives without cost sharing." 573 U. S., at 727. Now, the Government concedes that it lacks a compelling interest in providing such access, Reply Brief in No. 19–454, p. 10, and this time, the Government is correct.

In order to show that it has a "compelling interest" within the meaning of RFRA, the Government must clear a high bar. In *Sherbert v. Verner*, 374 U. S. 398 (1963), the decision that provides the foundation for the rule codified in RFRA, we said that "[o]nly the gravest abuses, endangering paramount interests," could "give occasion for [a] permissible limitation" on the free exercise of religion. *Id.*, at 406. Thus, in order to establish that it has a "compelling interest" in providing free contraceptives to all women, the Government would have to show that it would commit one of "the gravest abuses" of its responsibilities if it did not furnish free contraceptives to all women.

If we were required to exercise our own judgment on the question whether the Government has an obligation to provide free contraceptives to all women, we would have to take sides in the great national debate about whether the Government should provide free and comprehensive medical care for all. Entering that policy debate would be inconsistent with our proper role, and RFRA does not call on us to express a view on that issue. We can answer the compelling interest question simply by asking whether *Congress* has treated the provision of free contraceptives to all women as a compelling interest.

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“[A] law cannot be regarded as protecting an interest ‘of the highest order’ . . . when it leaves appreciable damage to that supposedly vital interest unprohibited.’” *Church of Lukumi Babalu Aye, Inc. v. Hialeah*, 508 U.S. 520, 547 (1993). Thus, in considering whether Congress has manifested the view that it has a compelling interest in providing free contraceptives to all women, we must take into account “exceptions” to this asserted “‘rul[e] of general applicability.’” *Gonzales v. O Centro Espírita Beneficente União do Vegetal*, 546 U.S. 418, 436 (2006) (quoting § 2000bb–1(a)). And here, there are exceptions aplenty. The ACA—which fails to ensure that millions of women have access to free contraceptives—unmistakably shows that Congress, at least to date, has not regarded this interest as compelling.

First, the ACA does not provide contraceptive coverage for women who do not work outside the home. If Congress thought that there was a compelling need to make free contraceptives available for all women, why did it make no provision for women who do not receive a paycheck? Some of these women may have a greater need for free contraceptives than do women in the work force.

Second, if Congress thought that there was a compelling need to provide cost-free contraceptives for all working women, why didn’t Congress mandate that coverage in the ACA itself? Why did it leave it to HRSA to decide whether to require such coverage *at all*?

Third, the ACA’s very incomplete coverage speaks volumes. The ACA “exempts a great many employers from most of its coverage requirements.” *Hobby Lobby*, 573 U.S., at 699. “[E]mployers with fewer than 50 employees are not required to provide” any form of health insurance, and a number of large employers with “‘grandfathered’” plans need not comply with the contraceptive mandate. *Ibid.*; see 26 U.S.C. § 4980H(c)(2); 42 U.S.C. § 18011. According to a recent survey, 13% of the 153 million Americans with employer-sponsored health insurance are enrolled in a grand-

fathered plan, while only 56% of small firms provide health insurance. Kaiser Family Foundation, *Employer Health Benefits: 2019 Annual Survey* 7, 44, 209 (2019). In *Hobby Lobby*, we wrote that “the contraceptive mandate ‘presently does not apply to tens of millions of people,’” 573 U. S., at 700, and it appears that this is still true apart from the religious exemption.¹⁰

Fourth, the Court’s recognition in today’s decision that the ACA authorizes the creation of exemptions that go beyond anything required by the Constitution provides further evidence that Congress did not regard the provision of cost-free contraceptives to all women as a compelling interest.

Moreover, the regulatory exemptions created by the Departments and HRSA undermine any claim that the agencies themselves viewed the provision of contraceptive coverage as sufficiently compelling. From the outset, the church exemption has applied to churches, their integrated auxiliaries, and associations. 76 Fed. Reg. 46623. And because of the way the accommodation operates under the Employee Retirement Income Security Act of 1974, the Departments treated a number of self-insured non-profit organizations established by churches or associations of churches, including religious universities and hospitals, as “effectively exempted” from the contraceptive mandate as well. Brief for Petitioners in No. 19–454, p. 4. The result was a complex and sometimes irrational pattern of exemptions.

The dissent frames the allegedly compelling interest served by the mandate in different terms—as an interest in providing “seamless” cost-free coverage, *post*, at 710–711, 723, 729 (opinion of GINSBURG, J.)—but this is an even weaker argument. What “seamless” coverage apparently means is cov-

¹⁰ In contrast, the Departments estimated that plans covering 727,000 people would take advantage of the religious exemption, and thus that between 70,500 and 126,400 women of childbearing age would be affected by the religious exemption. 83 Fed. Reg. 57578, 57581.

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erage under the insurance plan furnished by a woman's employer. So as applied to the Little Sisters, the dissent thinks that it would be a grave abuse if an employee wishing to obtain contraceptives had to take any step that would not be necessary if she wanted to obtain any other medical service. See *post*, at 725–726. Apparently, it would not be enough if the Government sent her a special card that could be presented at a pharmacy to fill a prescription for contraceptives without any out-of-pocket expense. Nor would it be enough if she were informed that she could obtain free contraceptives by going to a conveniently located government clinic. Neither of those alternatives would provide “seamless coverage,” and thus, according to the dissent, both would be insufficient. Nothing short of capitulation on the part of the Little Sisters would suffice.

This argument is inconsistent with any reasonable understanding of the concept of a “compelling interest.” It is undoubtedly convenient for employees to obtain all types of medical care and all pharmaceuticals under their general health insurance plans, and perhaps there are women whose personal situation is such that taking any additional steps to secure contraceptives would be a notable burden. But can it be said that all women or all working women have a compelling need for this convenience?

The ACA does not provide “seamless” coverage for all forms of medical care. Take the example of dental care. Although lack of dental care can cause great pain and may lead to serious health problems, the ACA does not require that a plan cover dental services. Millions of employees must secure separate dental insurance or pay dentist bills out of their own pockets.

In short, it is undoubtedly true that the contraceptive mandate provides a benefit that many women may find highly desirable, but Congress's enactments show that it has not regarded the provision of free contraceptives or the furnishing of “seamless” coverage as “compelling.”

Least restrictive means. Even if the mandate served a compelling interest, the accommodation still would not satisfy the “exceptionally demanding” least-restrictive-means standard. *Hobby Lobby*, 573 U.S., at 728. To meet this standard, the Government must “sho[w] that it lacks other means of achieving its desired goal without imposing a substantial burden on the exercise of religion.” *Ibid.*; see also *Holt v. Hobbs*, 574 U.S. 352, 365 (2015) (“[I]f a less restrictive means is available for the Government to achieve its goals, the Government must use it”).

In *Hobby Lobby*, we observed that the Government has “other means” of providing cost-free contraceptives to women “without imposing a substantial burden on the exercise of religion by the objecting parties.” 573 U.S., at 728. “The most straightforward way,” we noted, “would be for the Government to assume the cost of providing the . . . contraceptives . . . to any women who are unable to obtain them under their health-insurance policies.” *Ibid.* In the context of federal funding for health insurance, the cost of such a program would be “minor.” *Id.*, at 729.¹¹

The Government argued that we should not take this option into account because it lacked statutory authority to create such a program, see *ibid.*, but we rejected that argument,

¹¹ In 2019, the Government is estimated to have spent \$737 billion subsidizing health insurance for individuals under the age of 65; \$287 billion of that went to employment-related coverage. CBO, *Federal Subsidies for Health Insurance for People Under Age 65: 2019 to 2029*, pp. 15–16 (2019). While the cost of contraceptive methods varies, even assuming the most expensive options, which range around \$1,000 a year, the cost of providing this coverage to the 126,400 women who are estimated to be impacted by the religious exemption would be \$126.4 million. See Kosova, National Women’s Health Network, *How Much Do Different Kinds of Birth Control Cost Without Insurance?* (Nov. 17, 2017), <http://nwhn.org/much-different-kinds-birth-control-cost-without-insurance/> (discussing contraceptive methods ranging from \$240 to \$1,000 per year); 83 Fed. Reg. 57581 (estimating that up to 126,400 women will be affected by the religious exemption).

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id., at 729–730. Certainly, Congress could create such a program if it thought that providing cost-free contraceptives to all women was a matter of “paramount” concern.

As the Government now points out, Congress has taken steps in this direction. “[E]xisting federal, state, and local programs,” including Medicaid, Title X, and Temporary Assistance for Needy Families, already “provide free or subsidized contraceptives to low-income women.” Brief for Petitioners in No. 19–454, at 27; see also 83 Fed. Reg. 57548, 57551 (discussing programs).¹² And many women who work for employers who have religious objections to the contraceptive mandate may be able to receive contraceptive coverage through a family member’s health insurance plan.

In sum, the Departments were right to conclude that applying the accommodation to sincere religious objectors violates RFRA. See *id.*, at 57546. All three prongs of the RFRA analysis—substantial burden, compelling interest, and least restrictive means—necessitate this answer.

III

Once it was apparent that the accommodation ran afoul of RFRA, the Government was required to eliminate the violation. RFRA does not specify the precise manner in which a violation must be remedied; it simply instructs the Government to avoid “substantially burden[ing]” the “exercise of religion”—*i. e.*, to eliminate the violation. § 2000bb–1(a); see also § 2000bb–1(c) (providing for “appropriate relief” in judicial suit). Thus, in *Hobby Lobby*, once we held that applica-

¹² The Government recently amended the definitions for Title X’s family planning program to help facilitate access to contraceptives for women who work for an employer invoking the religious and moral exemptions. See 84 Fed. Reg. 7734 (2019). These definitions now provide that “[f]or the purpose of considering payment for contraceptive services only,” a “low income family” “includes members of families whose annual income” would otherwise exceed the threshold “where a woman has health insurance coverage through an employer [with] a sincerely held religious or moral objection to providing such [contraceptive] coverage.” 42 CFR § 59.2(2) (2019).